Every day, in public and private hospitals, doctors and patients discuss surgery.

It is one of the biggest steps in life.



What do patients usually receive as support?

A leaflet.



POLARIS PATIENT INFORMATION

Flexible Cystoscopy

A flexible cystoscopy is a telescopic inspection of your bladder and urethra, carried out in an outpatient clinic using only a local anaesthetic gel. This pathway guides you through what to expect before, during, and after the procedure, drawing on current guidance from the British Association of Urological Surgeons (BAUS), the European Association of Urology (EAU), and NICE.

VERSION
Standard

CONTENTS
Complete leaflet

Your pathway

What is a flexible cystoscopy?
What will happen during the procedure
Getting ready for your cystoscopy
After your cystoscopy and results
Common questions

PHASE 1

What is a flexible cystoscopy?

Before your procedure, it helps to understand exactly what a flexible cystoscopy is, why it has been recommended for you, and why the flexible technique is used in preference to other approaches.

What is a cystoscopy?

A telescopic look inside your bladder and urethra, carried out in a clinic.

A cystoscopy is a procedure that allows your surgeon to look directly inside your bladder — the organ that stores urine — and your urethra (the tube through which urine leaves your body). The word comes from the Greek for bladder (cystis) and viewing (skopein). The instrument used is called a cystoscope: a thin, flexible, fibre-optic tube with a small camera and light at its tip.

During the procedure, the cystoscope is passed gently along the urethra and into the bladder. Sterile water is run in slowly through the scope to expand the bladder slightly, which allows the surgeon to see the full lining clearly. The inside of the bladder appears on a monitor screen in the procedure room, and your surgeon can inspect it in real time.

What can a cystoscopy detect?

A flexible cystoscopy allows your surgeon to see the entire inner surface of the bladder and the lining of the urethra. It can identify:

- Haematuria (blood in the urine) — including visible haematuria (blood you can see) and non-visible haematuria (blood detectable only on urine testing)
- Bladder tumours or suspicious areas of the lining that may need a biopsy (a small tissue sample)
- Inflammation or abnormal changes to the bladder lining
- The position and condition of the ureteric orifices — the openings where the two ureters (the tubes carrying urine from the kidneys) drain into the bladder
- Ureteric stents (small internal drainage tubes placed between the kidney and bladder after kidney stone treatment or surgery)
- Features associated with lower urinary tract symptoms — for example, an enlarged prostate pressing on the outlet of the bladder
- Evidence of recurrent urinary tract infections

If you are attending for bladder cancer surveillance after a high-grade tumour, it is important to know that flexible cystoscopy using standard white-light inspection cannot reliably detect all types of disease. A condition called carcinoma-in-situ (CIS) — a flat, high-grade cancer that grows along the surface lining — can appear as a faint reddish patch or look entirely normal at white-light cystoscopy. The EAU 2024 guidelines note that a visually normal inspection does not exclude CIS in patients with a high-grade tumour history. This is why urine cytology (analysis of cells shed into the urine) is taken at the same visit in high-grade surveillance patients, and why additional techniques such as blue-light cystoscopy or narrow-band imaging (NBI) exist. Your surgeon will discuss which approach is appropriate for you.

Why flexible rather than rigid cystoscopy?

Flexible cystoscopy is performed in clinic under local anaesthetic — no general anaesthetic or theatre is required.

There are two types of cystoscope: flexible and rigid. Understanding the difference will help you appreciate why your surgeon has recommended the flexible approach for your procedure.

The flexible cystoscope

A flexible cystoscope is a thin, pliable, fibre-optic instrument — typically around the width of a pencil. Its flexible tip can be steered in different directions, which allows the surgeon to inspect all parts of the bladder lining without the instrument needing to be repositioned awkwardly. Because the instrument is slender and

Often long, maybe hard to read, and sometimes out of date.

What is a flexible cystoscopy?
What will happen during the procedure
Getting ready for your cystoscopy
After y
Comm

PHASE 1

What is a flexible cystoscopy?

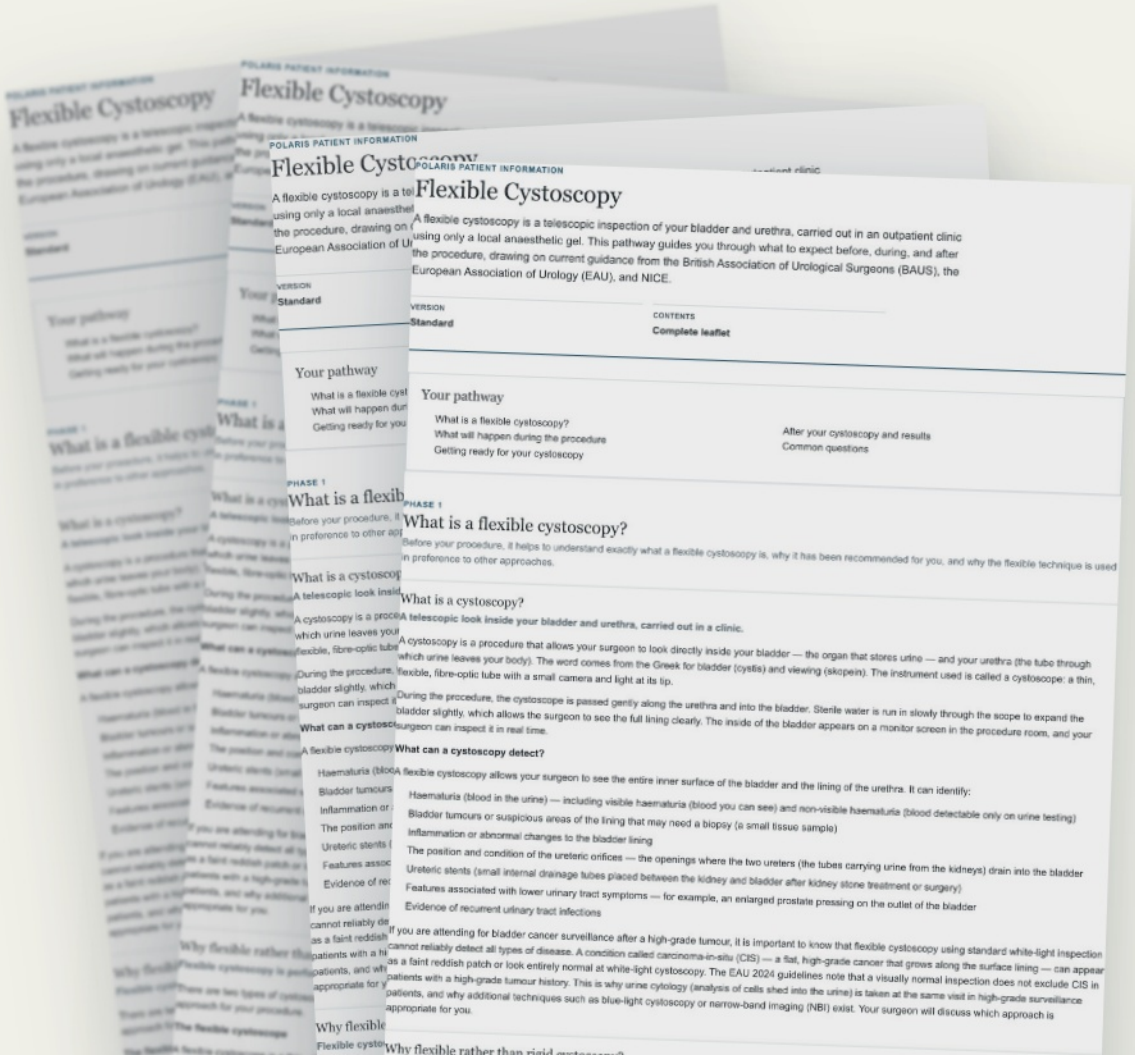
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